

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: NancyPeriod: April 25 to July 24, 2024Report Date: July 25, 2024Coverage: 1315/2184h (60%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

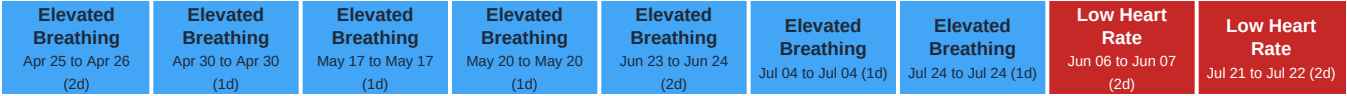
High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 91 days from Apr 25 to Jul 24

HR

Breathing



Last 24 Hours

Last 24 Hours: NORMAL30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	69 bpm	49 bpm	90 bpm
Breathing	17 brpm	10 brpm	39 brpm

Limited data in the final 24h (14/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 24	1 AM to 2 AM	1h	1	Elevated Breathing	24	18	39	Check SpO2 and lung sounds; assess for fluid overload or early infection

19 episodic events Detected, spanning 19h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
19	19h	Low Heart Rate	<1	60%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 06	9 PM to 10 PM	2h	2	Low Heart Rate	45	42	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 21	3 AM to 4 AM	7h	7	Low Heart Rate	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 25	9 PM to 10 PM	3h	3	Elevated Breathing	25	23	26	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 30	9 PM to 10 PM	1h	1	Elevated Breathing	25	22	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 17	4 PM to 5 PM	1h	1	Elevated Breathing	27	27	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 20	12 PM to 1 PM	1h	1	Elevated Breathing	24	24	25	Check SpO2 and lung sounds; assess for fluid overload or early infection

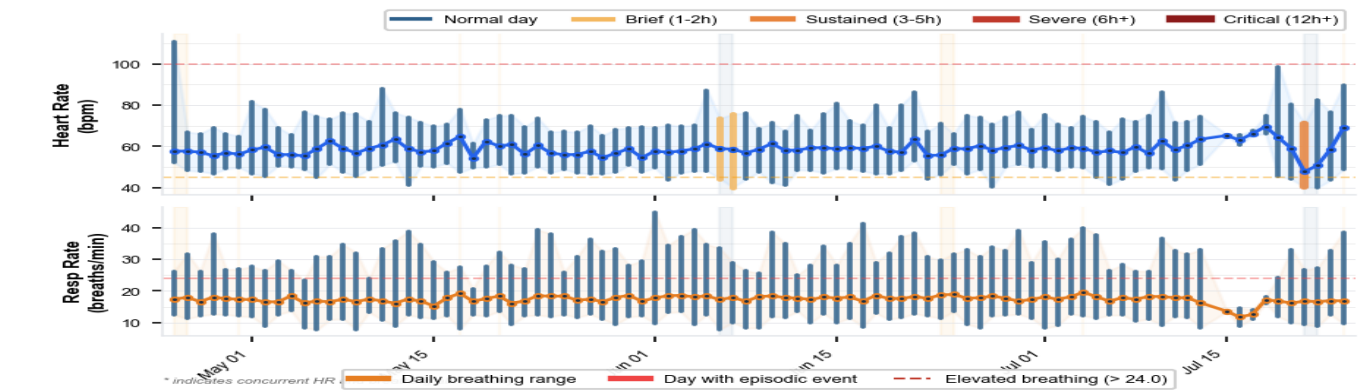
+ 3 additional condition(s) across Elevated Breathing; details in monitoring data.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 26 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 9 unstable phases with 18 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

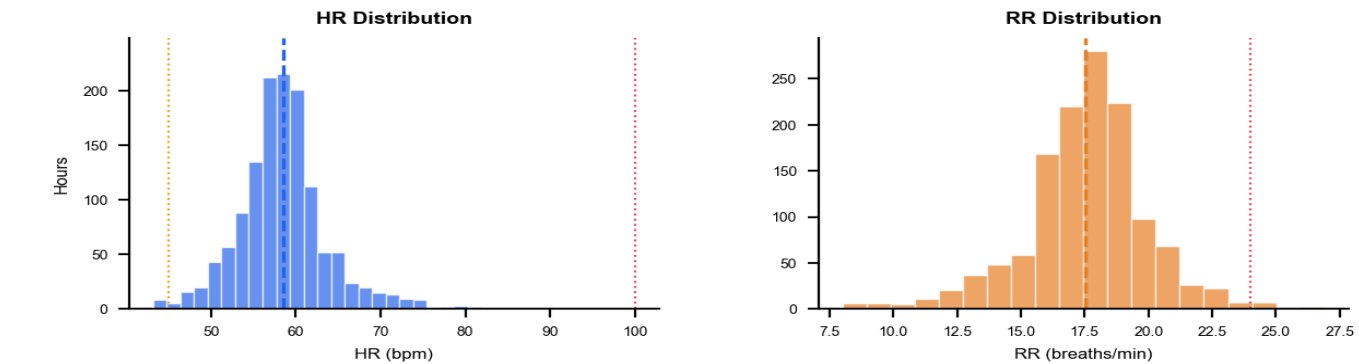
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 3 of 91 days (3%).
- 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

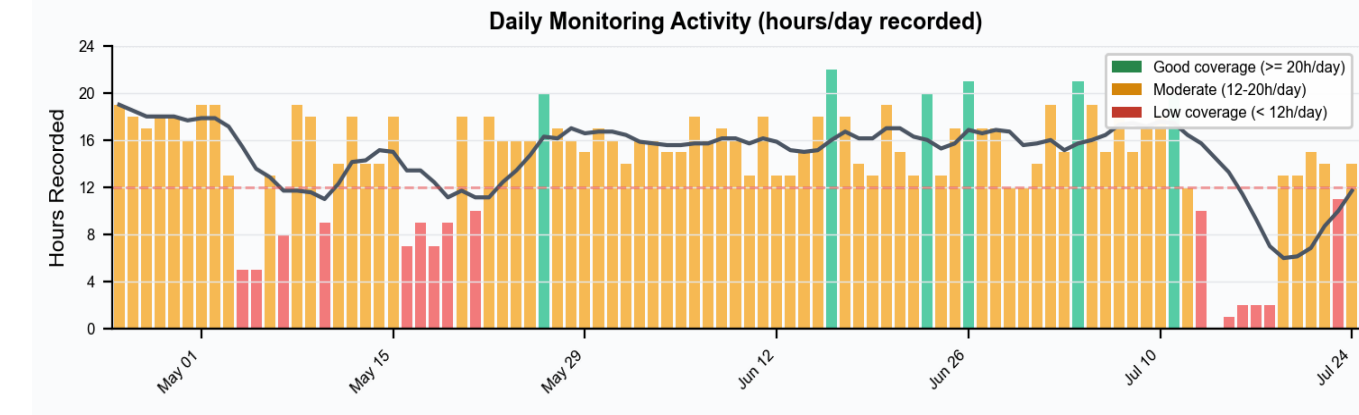


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.